SUMMARY OF A CONVERSATION, EL PASO, OCTOBER 2026

Six months of free decisions.

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Your new center, targeted to open around June 2027 (April at the earliest), with the MRI signed, no staff hired, and no systems chosen. The build starts around March 2027 so prior authorization is working before you open. Every operational automation decision is cheap and reversible today and will not be once the vendors are signed.

> **The free move, whatever you decide about me.** Put four terms in every vendor contract before signing: named API access to your own data, HL7 interface count and fee schedule fixed at signature, data export rights in standard formats at no charge, and DICOM query permissions for systems you authorize. Before signature these are contract terms. After signature they are hostage terms.

The candidates, ranked by value, highest first

#	PROCESS	RECOMMENDATION	WHY IT RANKS HERE
1	Contract terms before signature	**Free**	The four terms above. They cost nothing today and cannot be reopened after signature. The only item with a hard deadline, and the one I earn nothing from.
2	Prior authorization and eligibility verification	**Build**	The largest administrative cost, and the one not asked about. Medicare Advantage 2024: 11.5% of denied prior auths appealed, 80.7% of appeals overturned (KFF). 40% of physicians staff someone exclusively for it (AMA 2025). Payer FHIR prior auth APIs required by 2027-01-01. A human signs every submission and every appeal.
3	Owner reporting, including an agent you can ask questions of	**Build**	Modality, reader, referrer, turnaround, revenue per slot, appeal rate, from the RIS and the billing system joined. Ask it in plain words and it answers from your own numbers. Read-only, no patient contact, smallest compliance footprint on this list.
4	Scheduling: cancellation backfill	**Build, small**	UC Irvine study: 22.3% of outpatient radiology exams cancelled versus 2.0% no-shows, 70.2% of cancellations patient-initiated. The empty slot is known in advance. A ranked waitlist on top of the RIS, with a per-patient rate governor for the TCPA cap.
5	Medical records: inbound requests and outbound prior studies	**Build, selectively**	The HIPAA 30-day clock runs from receipt. Retrieval labor is not chargeable, only copying is. A new center opens with zero priors on file, and no vendor has solved outbound retrieval for a small site.
6	Radiology report assistance	**Build (exists)**	RadAssist, used daily by Dr. Ramos. Next on the roadmap: voice dictation, and spine, brain, and cardiac CTA (CAD-RADS) modules. Reads the scanner's numbers, drafts the report, and the radiologist confirms every value and signs. Nothing clinical leaves without a signature. At the center: DICOM worklist and query in, HL7 v2 ORU out to the RIS. A different build from records retrieval (row 5). One instance per center, each center's data its own; it costs more, by design, for governance.
7	Scheduling: reminders, self-scheduling, intake	**Buy (your RIS)**	Commodity, bundled into modern cloud imaging RIS products. Text reminders moved MRI no-shows from 5.1% to 3.8% in a randomized trial. Real, modest, already in the box.
8	Answering patients (inbound voice)	**Buy, I integrate**	26.5% of El Paso County residents speak English less than "very well," nearly all Spanish speakers. Highest-blast-radius failure on this list, and a funded vendor category. I specify, evaluate, integrate, and supervise.
9	Billing, claims, and the eligibility feed	**Buy**	Commodity clearinghouse function, established vendor market. I do not touch the claims work, and I am not building an eligibility system. Your billing vendor already exposes the EDI 270/271 eligibility feed. Row 2 builds the thing that reads that feed and acts on it, so the four access terms belong in the billing contract too.
10	Anything that touches the image clinically	**Declined**	Not from me, not from anyone, without an FDA pathway.

What you own if I disappear

- The specification, written for an engineer who has never met me.
- The code specific to this center, in a repository under your organization's account. Existing product of mine that I extend for you, under a perpetual license with the source in your hands.
- Runbooks for every operational procedure and vendor contact.
- Standard interfaces only: HL7 v2, DICOM, payer APIs, a named model vendor under its own BAA.
- Your RIS and voice vendor contracts in your name, not routed through me.

What I recommend you do not buy from me

- A custom voice agent. Buy a vendor.
- Reminders and intake. Use the RIS.
- Anything built to resell to other centers. This center is the reference implementation and the priority; your service terms do not change if I ever take on a second site.
- Anything that touches the image clinically.

Sources: KFF, Medicare Advantage prior authorization determinations in 2024 (2026-01-28), insurer data reported to CMS, all services, not imaging-specific. AMA 2025 Prior Authorization Physician Survey, n=1,000, self-reported. CMS-0057-F, 89 FR 8758. Liu et al., J Am Coll Radiol 2017, PMID 28583321, randomized. Aijaz et al., Acad Radiol 2024, PMID 38705764, UC Irvine, single center. ACS 2024 5-year, Table B16004, El Paso County. 45 CFR 164.524. 21 U.S.C. 360j(o)(1)(A): administrative support software, including appointment schedules, claims or billing information, business analytics, and determination of health benefit eligibility, is excluded from the device definition.

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Live proof: **radassist.app**, used daily by Dr. Ramos

What an AI agent is, and how it works in your imaging center.

Two kinds of agent

One that runs a workflow. An operations agent works one process from start to finish: a prior authorization, a cancelled slot filled from the waitlist, a records request. It works through your RIS and the payers' own electronic connections, not by clicking around a screen. Someone on your staff approves anything that leaves the building.

One you talk to. An owner's agent you text or call for answers from your own numbers. Every Monday it sends a short report: volumes by modality, reader, and referrer; authorization and denial rates; reconciliation.

The guardrails

- A person signs anything clinical and anything outbound.
- Numbers are calculated and checked, never guessed.
- HIPAA business associate agreements with every vendor that touches patient data.
- Patient data stays in the United States.

One order through prior authorization

1. The order arrives from the referring office and lands in your RIS.
2. The agent checks coverage and whether this payer requires authorization for this study.
3. If it does, the agent assembles the request from the order and clinical notes and checks it against the payer's rules.
4. Your staff member reviews and approves. Only then does it go to the payer, electronically.
5. The agent watches the payer's decision clock and schedules the study when approval arrives.
6. If denied, it drafts the appeal for a person to review and sign.

You will click through this yourself: a working prior authorization demo on synthetic data, no real patients, delivered in the first phase. rfinlay.github.io/imaging-priorauth-demo

What the dates depend on

The build starts around March 2027 so prior authorization works before you open. That holds when these are in place: a signed agreement; your RIS and PACS vendors selected, with sandbox access to their API, HL7, and DICOM interfaces; test access to the payers' FHIR and EDI connections; HIPAA business associate agreements signed; one named person on your side who approves; and your expected volumes and payer mix before payer-specific design starts. **If one of these slips, the dates move with it.**

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